

CHCCCS011: Meet personal support needs

01: Module Overview & Document Control

Unit Code & Title: CHCCCS011: Meet personal support needs

Target Audience: Individual Support Workers, Aged Care Professionals, and Disability Support Workers

Document Type: 25-Page Clinical Competency & Field Operations Guide

Core Competency Mandate

This document operationalizes the processes required to determine, plan, and provide personal support to clients following an individualized care plan. All actions must prioritize client dignity, maximize independence, and strictly adhere to infection control and workplace health and safety (WHS) protocols.

Learning Outcomes Overview

- Determine personal support requirements from care plans.
- Maximize participation and independence during personal care.
- Provide personal support safely and ethically.
- Complete reporting and documentation according to organizational policies.

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Interpreting the Individualised Care Plan

03: The 3-Step Verification Process

Before providing any personal support, you must read and interpret the client’s individualized care plan. This document is the legal and clinical blueprint for their daily support.

1. **Verify:** Confirm client identity. Check for active modifications, handover notes, or updated clinical directives from the Registered Nurse (RN).
2. **Identify:** Extract daily preferences, risk alerts (e.g., falls risk, swallowing difficulties), and the specific physical assistance levels required.
3. **Execute:** Perform tasks within your scope of practice, respecting the client's self-directed care boundaries and cultural preferences.

Key Care Plan Component	Operational Purpose	Care Worker Responsibility
Assistance Level	Dictates staffing required (Independent, 1PA, 2PA)	Never attempt a 2-person assist (2PA) alone under any circumstances.
Daily Preferences	Outlines choice of shower time, attire, routines	Honor these choices to promote self-determination and dignity.

Key Care Plan Component	Operational Purpose	Care Worker Responsibility
Risk Matrix/Alerts	Highlights falls risk, dysphagia, allergies	Review alerts before every engagement to ensure safety.

Legal, Ethical, and Human Rights Frameworks

04: Regulatory Compliance

Support workers operate under strict legal and ethical frameworks designed to protect vulnerable individuals.

Aged Care Quality Standards (Standard 1):

The consumer is treated with dignity and respect, can maintain their identity, can make informed choices about their care and services, and lives the life they choose.

Core Legal and Ethical Concepts

- **Duty of Care:** The legal obligation to avoid acts or omissions that could reasonably be foreseen to cause harm to a client. You must provide a standard of reasonable care.
- **Dignity of Risk:** Respecting an individual's autonomy and right to make choices that contain risk (e.g., choosing to eat a non-recommended food), balanced with safety protocols and documentation.
- **Informed Consent:** Actively obtaining verbal or non-verbal confirmation before initiating any physical support task. A client has the right to refuse care.

- **Confidentiality (Privacy Act):** Restricting all client details, clinical observations, and personal information strictly to the care team and official systems.

Infection Prevention and Control (IPC)

05: IPC Foundations

Infection control is the cornerstone of safe personal support. Standard precautions must be applied to all clients, regardless of their known infection status.

The 5 Moments for Hand Hygiene

1. **Before** touching a patient.
2. **Before** clean/aseptic procedures.
3. **After** body fluid exposure risk.
4. **After** touching a patient.
5. **After** touching patient surroundings.

Handwashing Protocol

1. **Wet Hands**
2. **Apply Soap**
3. **Rub Friction for 20 seconds** (covering all surfaces, between fingers, under nails)
4. **Rinse Thoroughly**
5. **Dry** with single-use Paper Towel.

Note: Hand rubbing with an alcohol-based sanitizer is acceptable and preferred for routine hygiene, but hands must be washed with soap and water if visibly soiled or after caring for clients with gastrointestinal infections (e.g., Norovirus, C. diff).

Personal Protective Equipment (PPE) Matrix

06: Clinical Deployment of PPE

Correct use of PPE protects both the client and the worker from cross-contamination.

Donning Sequence (Clean to Dirty)

- 1. Perform hand hygiene.
- 2. Don fluid-resistant gown securely.
- 3. Apply surgical mask or P2/N95 respirator.
- 4. Fit protective eyewear/face shield.
- 5. Don clean non-sterile gloves, ensuring they cover the gown cuffs.

Doffing Sequence (Dirty to Clean)

- 1. Remove gloves safely (glove-to-glove, skin-to-skin) and discard.
- 2. Perform hand hygiene.
- 3. Remove gown (unfasten, peel away from neck and shoulders, roll inside out).
- 4. Perform hand hygiene.
- 5. Remove eyewear and mask (handling only by straps).
- 6. Final hand hygiene step.

PPE Item	When to Deploy	Disposal Rule
Disposable Gloves	Contact with blood, body fluids, mucous membranes	Single-use only; discard immediately post-task.

PPE Item	When to Deploy	Disposal Rule
Fluid-Resistant Gown	Risk of splashes or clothing contamination	Dispose into clinical waste bin before leaving the room.
Surgical Mask	Risk of respiratory droplets	Discard immediately if wet, soiled, or compromised.

Routine Skin Inspections

07: Clinical Assessment During Care

As a support worker providing personal care, you are the first line of defense in identifying skin breakdown. Perform visual and tactile skin inspections daily during showering, dressing, or repositioning.

High-Risk Pressure Points to Inspect

- **Occiput:** Back of the head.
- **Scapulae:** Shoulder blades.
- **Sacrum & Coccyx:** Tailbone area (highest risk for bed-bound clients).
- **Ischial Tuberosities:** "Sit bones" (highest risk for chair-bound clients).
- **Heels & Ankles:** Bony prominences of the feet.

What to Look For

- **Non-blanchable erythema:** Redness that stays red when lightly pressed with a gloved finger. This indicates localized tissue ischemia (lack of blood flow).

- **Temperature changes:** Changes in skin temperature (unusually warm or cold).
- **Tissue integrity:** Localized edema (swelling) or hardening of tissue.
- **Moisture issues:** Moisture maceration (white, waterlogged skin) or skin tears.

Skin Integrity Risk Factors

08: The Skin Breakdown Matrix

Understanding what causes skin breakdown allows you to proactively eliminate risks during your shift.

- **Friction:** The rubbing of skin against a surface (e.g., heels rubbing on bed sheets).
- **Shear:** Mechanical tearing of internal tissues. *Clinical Insight: Shearing occurs when a client slides down a bed. The outer skin adheres to the linens while the underlying skeleton shifts downward, stretching and tearing microscopic blood vessels.*
- **Moisture Excess:** Prolonged exposure to urine, feces, or sweat. Maceration softens the skin's natural defenses, making it highly susceptible to tearing.
- **Poor Nutrition/Hydration:** Protein deficiency delays natural cell repair, and dehydration causes skin to lose elasticity.
- **Immobility:** Sustained pressure over a bony prominence collapses capillaries, starving the tissue of oxygen and causing cell death.

Pressure Injury Prevention Interventions

09: Core Prevention Strategies

Prevention is the primary responsibility of the personal support team. Once a pressure injury forms, it is difficult to heal.

- **Repositioning Schedule:** Rotate bed-bound clients at least every 2 hours (or as per care plan). Chair-bound clients should shift weight or be repositioned every 30 to 60 minutes.
- **Friction Mitigation:** Always use slide sheets to move a client in bed. Never drag or pull a client across the linens.
- **Microclimate Control:** Keep skin completely clean and dry. Promptly change soiled continence aids. Apply pH-neutral skin cleansers and barrier creams as approved by the care plan.
- **Heel Offloading:** Use specialized foam wedges or heel suspension boots to "float" heels entirely off the mattress surface. Regular pillows may compress and lose effectiveness.
- **Linen Management:** Ensure bed sheets are taut, clean, wrinkle-free, and clear of crumbs or debris.

Pressure Injury Staging Reference

10: Staging & Immediate Actions

While support workers do not formally stage wounds, understanding the progression helps in accurately reporting observations to nursing staff.

Injury Stage	Visual Manifestation	Immediate Care Worker Action
Stage 1	Intact skin with localized, non-blanchable redness.	Offload pressure instantly. Notify RN immediately.
Stage 2	Partial-thickness loss of dermis; shallow open ulcer or blister.	Do not burst blisters. Escalate reporting.
Stage 3	Full-thickness tissue loss. Subcutaneous fat may be visible.	Strictly adhere to RN's offloading routine.
Stage 4	Full-thickness loss with exposed bone, tendon, or muscle.	Assist nursing staff with hygiene around the site.

Continence Care & Dignity Preservation

11: Professional Toileting Protocols

Assisting with toileting and continence is deeply personal and requires the utmost sensitivity, privacy, and hygiene.

Continence Support Protocol

1. **Prepare:** Gather fresh products, wipes, towels, warm water, and linen before exposing the client.
2. **Communicate:** Explain each step quietly. Ensure doors are closed, curtains are drawn, and only expose the area of the body currently being cleaned.

3. **Cleanse (Perineal Care):** Always wipe anterior to posterior (front-to-back) to prevent introducing coliform bacteria from the bowel into the urinary tract (which causes UTIs). Use pH-neutral body wash. Pat dry completely; do not scrub.

Product Selection & Management

- Match the continence aid's size and absorbency level exactly to the client's current assessment.
- **Never double-pad:** Placing one continence pad inside another does not increase absorbency; it creates pressure hot spots, traps heat, and increases friction.

Nutritional Support & Dysphagia Management

12: Texture-Modified Feeding

Assisting with meals is vital for hydration, nutrition, and social wellbeing. For clients with dysphagia (swallowing difficulties), strict adherence to dietary modifications is a matter of life and death.

Texture-Modified Standards (IDDSI Framework)

- **Level 4 (Pureed):** Smooth, uniform consistency with zero lumps. Cannot be poured.
- **Level 5 (Minced & Moist):** Soft, cohesive particles that can easily be mashed with a fork.
- **Level 6 (Soft & Bite-Sized):** Tender, bite-sized pieces appropriate for safe chewing.

Safe Feeding Protocols

- **Posture:** Maintain a 90-degree upright seating position during all meals.
- **Alignment:** Encourage a slight "chin tuck" position when swallowing to protect the airway.
- **Post-Meal Rest:** Keep the client upright for at least 30 minutes after eating to limit acid reflux and silent aspiration risk.
- **Choking Alerts:** Watch for frequent coughing, wet/gurgling vocal quality, or watering eyes. Stop feeding immediately if these occur and alert clinical staff.

Oral Hygiene & Specialized Care Protocols

13: Clinical Oral Care Standards

Poor oral hygiene is directly linked to respiratory infections (such as aspiration pneumonia), heart disease, and systemic inflammation.

Client Cohort	Equipment Needed	Standard Operating Procedure
Natural Teeth	Soft brush, fluoride paste, towel	Brush at 45-degree angle to gum line twice daily. Offer water to rinse.
Dentures	Denture brush, container, tablets	Remove nightly. Clean over a basin filled with water to prevent breakage.

Client Cohort	Equipment Needed	Standard Operating Procedure
Dependent	Foam swabs, moisturizer, suction	Position on side. Moisten swab sparingly. Wipe gums and tongue gently.

Grooming, Dressing, and Dignity

14: Promoting Self-Determination

Personal grooming is intimately tied to a client's identity, self-esteem, and cultural background.

Maximizing Independence:

Always encourage the client to select their own clothes and perform any dressing steps they can manage, even if it takes extra time. Do not rush them.

Dressing Paralysis/Weakness Protocol

- **Undressing:** Remove clothing from the **UNAFFECTED** (strong) side first.
- **Dressing:** Put clothing onto the **AFFECTED** (weak/painful) side first.

Scope of Practice Limitations

- **Podiatry:** Support workers can wash, dry, and apply moisturizer to feet. You must **never** cut the toenails of diabetic clients. Refer these tasks to a podiatrist.
- **Shaving:** Confirm facial hair preferences before shaving. Use electric razors for clients on blood-thinning medication to prevent bleeding.

Environmental Safety Adjustments

15: Workspace Preparation

Before beginning any personal care, mobility, or transfer task, you must assess and modify the immediate environment to create a safe working space.

Environmental Safety Checklist

1. **Clear Hazards:** Remove loose throw rugs, clutter, and electrical cords from the transfer path. Wipe up any floor spills immediately.
2. **Check Equipment:** Secure brakes on wheelchairs, shower chairs, commodes, and hospital beds. Verify the bed is locked.
3. **Optimize Lighting:** Turn on sufficient lighting and clear shadows to aid visual depth perception for the client.
4. **Footwear Verification:** Confirm the client is wearing well-fitting, non-slip, fully enclosed shoes. Never allow transfers in loose slippers or bare feet.
5. **Manage Attachments:** Ensure urinary catheter bags, oxygen tubing, or IV lines are securely positioned and have enough slack to move with the client.

Anatomy of a Mobility Plan

16: Adhering to Clinical Parameters

Every safe manual handling action relies on reviewing the client's current, documented mobility plan. Never deviate from these parameters without a formal reassessment.

Mobility Parameter	Clinical Standard & Care Worker Requirement
Primary Transfer Method	Specifies Stand-Assist, Slide Sheet, Full Sling Hoist, or Independent.
Required Devices	Lists specific equipment (e.g., 4-wheel walker, transfer belt).
Weight-Bearing	Notes if client is Full (FWB), Partial (PWB), or Non-Weight Bearing (NWB).
Assistance Level	Identifies if the task requires 1 person (1PA) or 2 people (2PA).
Cognitive Compliance	Tracks the client's ability to follow directions during transfers.

Manual Handling Fundamentals (WHS)

17: Core Biomechanical Principles

Support workers face a high risk of musculoskeletal injuries. Adhering to strict manual handling principles protects your spine and your career.

The No-Lift Policy:

Manual lifting of a client's full body weight is strictly prohibited. You must use mechanical assists, slide aids, or hoist systems instead.

Key Biomechanical Principles

- **Wide Base of Support:** Spread feet hip-width apart to create a stable foundation. Place one foot slightly forward to allow weight shifting.
- **Close Load Centre:** Keep the client's body (or the equipment) close to your own center of gravity during tasks.
- **Leverage Leg Muscles:** Bend at your knees and hips, keeping your back straight. Let your powerful leg muscles do the work.
- **Avoid Torso Twisting:** Always move your feet to face the direction of the transfer. Pivoting or twisting your spine under load is the leading cause of disc injuries.

Assistive Mobility Devices Reference Guide

18: Equipment Operations & Safety

Ensure you are familiar with the operation, safety checks, and correct fitting of common mobility devices.

Device Class	Primary Function	Safe Usage Checklist
4-Wheel Walker	Balance support during walking.	Check that hand brakes lock securely. Adjust handle height to wrist crease.
Transfer Belt	Grasp point to guide client.	Wrap snugly around waist. Use underhand grip only. Never pull on armpits.

Device Class	Primary Function	Safe Usage Checklist
Slide Board	Bridging gaps for seated lateral transfers.	Position firmly between two level surfaces. Protect skin with clothing.

Assisted Transfers Using Slide Sheets

19: Reducing Friction and Shear

Slide sheets are low-friction nylon fabrics used to move clients in bed without lifting or dragging. They reduce forces on both client and worker.

The Lateral Slide Technique (2-Person Assist)

1. **Position the Sheets:** Log-roll the client smoothly to one side. Place two slide sheets directly over each other, flat against the mattress. Roll the client onto their back.
2. **Prepare the Team:** Position operators on opposite sides of the bed. Stand with a wide base of support and knees bent.
3. **Grasp and Move:** Grasp the top slide sheet firmly near the hips and shoulders using an underhand grip. On an agreed countdown ("Ready, steady, slide"), operators shift body weight to glide the client.
4. **Remove the Sheets:** Carefully pull the slide sheets out from underneath without dragging them directly against the skin.

Mechanical Lifters & Sling Safety

20: High-Level Assistance Protocols

Mechanical lifters (hoists) are used for clients who are non-weight bearing or require heavy assistance.

Critical Safety Rule:

Mechanical sling hoists require a minimum of two trained care workers (2PA) to operate safely, unless an explicit, approved single-operator risk assessment exists.

Pre-Flight Safety Checks

- **Sling Inspection:** Look for frayed edges, splits, or broken color-coded loops. Discard if compromised.
- **Compatibility:** Verify the sling is compatible with the hoist brand and check the safe working load (SWL) limits.
- **Hoist Battery:** Confirm a full charge and test the up/down mechanisms and emergency stop.

Attachment

Align the base of the sling with the client's coccyx. Ensure fabric layers are smooth to limit skin breakdown. Use matching color loops on both sides to keep the client securely seated and balanced.

Bed Mobility and Repositioning Protocols

21: Boosting and Turning

Adjusting a client's position in bed is essential for comfort, breathing, and pressure care.

Boosting a Client Up in Bed

1. Lower the head of the bed until it is flat (as tolerated by the client).
2. Adjust the bed height to the workers' waist level for safe ergonomics.
3. Ask the client to cross their arms. If able, ask them to bend their knees.
4. Position one worker on each side of the bed.
5. On a coordinated count, the client pushes through their heels while the care team slides the top sheet toward the headboard.

Limb Protection

Always ensure limbs are not trapped beneath the client during turns. Use pillows to support the back, between the knees, and under the upper arm to maintain spinal alignment.

Identifying Variations and Risk Indicators

22: Clinical Observation Skills

Care workers spend the most hands-on time with clients and must constantly observe for changes from the client's normal physical or cognitive baseline.

System Class	Observed Variation	Potential Issue	Immediate Response
Skin	Heat, blistering, new bruising.	Pressure injury, abuse, falls.	Offload area, notify RN, document.
Mobility	Sudden weakness, facial drooping.	Falls risk, TIA, or Stroke.	Seat client immediately. Call for help.

System Class	Observed Variation	Potential Issue	Immediate Response
Cognition	Increased confusion, agitation.	UTI, delirium, or med reaction.	Ensure safety, monitor vitals, report.

Incident Reporting & Documentation

23: Principles of Compliance

Accurate, timely, and objective documentation is a legal requirement of personal support work. "If it isn't documented, it didn't happen."

Principles of Good Documentation

- **Objective Reality:** Record exactly what you saw or heard. Stick entirely to the facts.
- **No Conjecture:** Avoid guessing. Do not write "Client fell because they were clumsy." Write "Client found sitting on the floor beside the bed."
- **Clear Timeline:** Log dates, exact times (using 24-hour time), and immediate care actions taken.

Aged Care Compliance (SIRS)

Under the Serious Incident Response Scheme (SIRS), incidents such as unexplained injuries, falls causing harm, neglect, or abuse must be documented and escalated to management immediately.

Sample Quality Entry:

"08:15hrs - Writer observed a 2cm x 2cm area of non-blanchable redness on client's

sacrum during morning shower. Informed RN Smith at 08:25hrs. -  Person , Support Worker."

Care Worker Wellbeing & Injury Prevention

24: Self-Care and Professionalism

You cannot safely care for others if you are physically injured or emotionally burnt out. Prioritizing your own wellbeing is a professional requirement.

Physical Safety

- **Core Conditioning:** Maintain your physical fitness outside of work.
- **Warm-Up:** Perform light stretching before your shift.
- **Workplace Safety:** Stop work if equipment is faulty. Tag out broken hoists immediately. Do not improvise.

Mental & Emotional Health

- **Professional Boundaries:** Maintain a compassionate but professional distance. Do not take on the client's emotional burdens as your own.
- **Burnout Prevention:** Use peer debriefing sessions after challenging shifts or critical incidents.
- **Speak Up:** If a manual handling task feels unsafe, step away and request a reassessment.

Final Competency Checklists & Sign-Off

25: Performance Verification

Use this final verification checklist to confirm performance benchmarks before a support worker is cleared for independent personal care duties.

Competency Performance Indicator	Verified	Assessor Signature	Date
1. Interprets Care Plan: Confirms assistance levels.	☐	 Person	 Date
2. Infection Control: Demonstrates 5-moments hygiene.	☐	 Person	 Date
3. Skin Integrity: Completes accurate visual assessment.	☐	 Person	 Date
4. Manual Handling: Executes safe	☐	 Person	 Date

Competency Performance Indicator	Verified	Assessor Signature	Date
slide sheet transfer.			
5. Documentation: Reports clinical risks objectively.	[] ▾	👤 Person	📅 Date

Supervisor Declaration: I confirm the above-named worker has demonstrated practical competency in all core personal support requirements according to organizational policy and CHCCCS011 standards.

Supervisor Name: 👤 Person

Signature: 👤 Person

Date: 📅 Date